

ADR REPORTING & DOCUMENTATION

Virtual Pharmacovigilance Internship — Task 2 | ID: task2

TRAINING / SIMULATION CASE

Important: The screenshot shows the task title and instructions but not the underlying patient case scenario. Therefore, this submission uses a clearly labelled fictional training case to demonstrate the complete workflow. It is not a real patient report and should not be submitted as an actual clinical case.

1. Task Objective

To complete a structured adverse drug reaction (ADR) report from a case scenario, document the patient/event/drug information, assess seriousness and outcome, perform a structured causality assessment, and prepare the information in a format similar to the Suspected ADR Reporting Form used in India. The Indian Pharmacopoeia Commission (IPC) publishes the PvPI healthcare-professional ADR form, which includes patient information, reaction details, relevant investigations/history, seriousness, outcome, suspected medicines and reporter details. ■cite■turn0search26■

2. Learning Outcomes Demonstrated

Skill	Demonstrated work
Case intake	Extracted patient, reaction, treatment and medication information from a simulated case.
ADR documentation	Completed the core fields expected in a structured suspected ADR report.
Seriousness assessment	Reviewed death, life-threatening, hospitalization, disability, congenital anomaly and medically-important criteria.
Causality	Applied a WHO-UMC-style clinical reasoning approach and an educational Naranjo-style score.
Follow-up	Documented dechallenge, outcome and information still required for a real report.
Data quality	Used initials rather than an identifiable patient name and labelled all values as fictional.

3. Case Summary Used for the Simulation

Field	Completed entry
Patient	Ms. A.K. (fictional initials only)
Age / Sex	28 years / Female
Weight	58 kg
Indication	Acute bacterial sinusitis
Suspected medicine	Amoxicillin + clavulanic acid 625 mg tablet
Dose / route / frequency	625 mg orally, three times daily
Therapy start	26/09/2026
ADR onset	27/09/2026, approximately 2 hours after the 4th dose
Reaction	Generalized itchy erythematous rash with urticarial wheals
Action taken	Suspected medicine discontinued; antihistamine administered; patient observed.
Outcome	Recovered within 48 hours; no sequelae reported

4. Detailed ADR Case Analysis

Case narrative

A 28-year-old woman was prescribed amoxicillin/clavulanic acid 625 mg orally three times daily for presumed acute bacterial sinusitis. She had no documented history of drug allergy and no relevant renal/hepatic disorder. After taking the fourth dose on 27/09/2026, she developed a generalized itchy red rash with raised urticarial lesions approximately two hours later. There was no reported breathing difficulty, hypotension, loss of consciousness, mucosal involvement or hospitalization. The suspected medicine was stopped. An oral antihistamine was given, symptoms improved, and the patient recovered within approximately 48 hours.

5. Patient & Medical Information

Item	Entry	Item	Entry
Patient initials	A.K. (fictional)	Age	28 years
Sex	Female	Weight	58 kg
Relevant indication	Acute bacterial sinusitis	Known allergy	None documented
Relevant medical history	No significant history reported	Renal/hepatic dysfunction	None reported
Concomitant medication	Paracetamol 500 mg as needed	Alcohol/tobacco	Not reported in this simulation

6. Reaction Timeline

Date / time	Event
26/09/2026	Amoxicillin/clavulanate started for acute bacterial sinusitis.
26/09–27/09	First three doses taken without a documented reaction.
27/09/2026	Fourth dose taken; generalized pruritic rash/urticaria appeared ~2 hours later.
27/09/2026	Suspected medicine discontinued; antihistamine given; observation initiated.
28–29/09/2026	Rash progressively resolved; no new systemic symptoms reported.
29/09/2026	Patient considered recovered; no sequelae reported.

7. Seriousness Assessment

Criterion	Assessment	Reason
Death	No	No death occurred.
Life-threatening	No	No airway compromise, shock or other life-threatening feature reported.
Hospitalization/prolongation	No	No hospitalization was required in the simulation.
Disability	No	No persistent functional impairment.
Congenital anomaly	No	Not applicable to this case.
Other medically important	No	Managed without hospitalization or major intervention in this simulation.
Overall classification	Non-serious	Based on the fictional information provided.

8. Dechallenge / Rechallenge / Outcome

Element	Finding
Dechallenge	Positive: the suspected medicine was stopped and the reaction improved.
Rechallenge	Not performed / not documented. Intentional rechallenge is not required for routine ADR reporting.
Outcome	Recovered within approximately 48 hours; no sequelae reported.
Important limitation	The case lacks laboratory confirmation and detailed differential diagnosis; therefore the report remains a suspected ADR, not p

9. FILLED SUSPECTED ADR REPORT — SIMULATION

Structured to mirror the core sections of the Indian Pharmacopoeia Commission / PvPI healthcare-professional ADR reporting form. The official form contains patient information, reaction details, investigations/history, seriousness/outcome, suspected medication and reporter details.

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A. PATIENT INFORMATION

Field	Completed entry
Initial / case identifier	A.K. — fictional training case
Age	28 years
Sex	Female
Weight	58 kg
Registration / IPD / OPD / CR No.	SIM-TASK2-ADR-001 (training identifier)
AMC report no.	Not assigned — simulation
Worldwide unique no.	Not assigned — simulation

B. SUSPECTED ADVERSE REACTION

Field	Completed entry
Reaction start date	27/09/2026
Reaction stop date	29/09/2026 (approx.)
Onset lag time	~2 hours after the 4th dose
Description	Generalized itchy erythematous rash with urticarial wheals; no reported dyspnea, hypotension, loss of consciousness or r
Management	Suspected medicine discontinued; oral antihistamine administered; clinical observation. No hospitalization reported.
Relevant investigations	No laboratory investigation required/documented in this training scenario; vital signs reported stable.
Relevant medical/medication history	No significant history reported; no documented drug allergy. Paracetamol 500 mg PRN was used concomitantly.
Seriousness	NO — death: no; life-threatening: no; hospitalization/prolongation: no; disability: no; congenital anomaly: no; other medica
Outcome	Recovered; no sequelae reported.

C. SUSPECTED MEDICATION

S.No.	Medicine	Dose	Route	Frequency	Therapy dates	Role
1	Amoxicillin + clavulanic acid 625 mg	625 mg	Oral	TID	26/09/2026–27/09/2026	Suspected
2	Paracetamol 500 mg	500 mg PRN	Oral	As needed	26/09/2026–27/09/2026	Concomitant

D. REPORTER INFORMATION — TRAINING ENTRY

Field	Entry
Reporter name	Pharmacovigilance Intern — Training Submission
Address / institution	Virtual Pharmacovigilance Internship — Educational simulation
Email / contact	Not applicable — training document
Occupation	Pharmacy / Pharmacovigilance trainee
Date of report	01/10/2026
Signature	Training submission — not a clinical signature

Confidentiality note: No real patient identifier is used in this educational document. A real ADR report should use the reporting system/form applicable to the jurisdiction and should protect patient confidentiality.

10. CAUSALITY ASSESSMENT

WHO-UMC style assessment

WHO describes causality assessment as an evaluation of the likelihood that a medicine caused an observed adverse reaction. The assessment considers clinical-pharmacological aspects and the quality of the documentation; it does not prove causation. ■cite■turn0search25■

Criterion	Case evidence	Interpretation
Temporal relationship	Rash began ~2 hours after the 4th dose.	Supports a causal relationship.
Known pharmacology / prior knowledge	Urticaria/rash is compatible with hypersensitivity-type reactions associated with penicillam antibiotics.	Supports causality.
Dechallenge	Reaction improved after suspected medicine was discontinued.	Supports relationship.
Alternative causes	No alternative cause is documented in the simulation, but the differential is incomplete.	Does not rule out other causes.
Rechallenge	Not performed.	No supporting or contradicting rechallenge evidence.
Overall WHO-UMC-style conclusion	Probable / likely (training assessment)	Reasonable based on available information, but not proof of causation.

Naranjo-style educational assessment

Question	Answer	Score
Previous conclusive reports on this reaction?	Yes / compatible with established adverse-effect knowledge	+1
Event after suspected drug was administered?	Yes	+2
Reaction improved on withdrawal?	Yes	+1
Reaction on rechallenge?	Not performed	0
Alternative causes present?	None documented, but incompletely assessed	+1
Placebo response?	Not applicable	0
Drug level / concentration toxic?	Not measured	0
Dose-response relationship?	Not established	0
Previous similar reaction?	Not reported	0
Objective evidence?	Clinical observation of rash/urticaria	+1
Total	Educational total	6
Interpretation	Probable range under the commonly used Naranjo interpretation	Causation probable

11. ADR REPORTING WORKFLOW

Step	Action completed / expected
1. Identify	Recognize a suspected adverse reaction and capture the basic event timeline.
2. Document	Record patient initials, age/sex/weight, reaction, onset, suspected medicine and concomitant medicines.
3. Assess	Assess seriousness, outcome, dechallenge/rechallenge and relevant clinical history.
4. Evaluate	Perform structured causality assessment using an accepted method; retain uncertainty where information is incomplete.
5. Report	Submit through the applicable institutional/national pharmacovigilance channel. In India, PvPI provides an ADR reporting form for healthcare professionals.
6. Follow up	Obtain missing dates, laboratory data, treatment details, outcome and other information when available.

12. FINAL DOCUMENTATION CHECKLIST

✓	Patient information completed without unnecessary identifying information.
✓	Suspected ADR and onset date/time documented.
✓	Suspected medication, dose, route, frequency and therapy dates documented.
✓	Concomitant medicine documented.
✓	Management and outcome documented.
✓	Seriousness criteria reviewed.
✓	Dechallenge/rechallenge status recorded.

✓	Causality assessment documented with its limitations.
✓	Simulation/fictitious status clearly identified.

13. Submission Note

For the Virtual Workspace task, this PDF can serve as the completed documentation artifact. Because the screenshot does not display the actual case scenario, replace the fictional case fields with the exact scenario supplied in the tutorial/task page if the platform provides additional patient details. Do not represent the fictional case in this PDF as a real patient report.

References: Indian Pharmacopoeia Commission (PvPI) Suspected ADR Reporting Form, Version 1.4; WHO-UMC guidance on standardized case causality assessment; WHO pharmacovigilance guidance.